

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 28

Age: 58 years old

Sex: Female

MRN: 000-TEST-28

Admission Date: [DATE]

Discharge Date: [DATE]

Length of Stay: 4 days

****PRIMARY DIAGNOSIS****

Type 2 Diabetes Mellitus with severe hyperglycemic episode (admission glucose 487 mg/dL). HbA1c on admission: 11.2% (prior baseline 9.8%). Acute diabetic complications including metabolic acidosis (pH 7.28, HCO₃ 16 mEq/L) requiring IV fluid resuscitation and insulin drip management.

****SECONDARY DIAGNOSES****

- Systolic Heart Failure, EF 38% (NYHA Class II–III)
- Obstructive Sleep Apnea (untreated)
- Hypertension
- Hyperlipidemia
- Stage II CKD (Cr 1.8, eGFR 41 mL/min)

****HOSPITAL COURSE****

Patient admitted via ED with polyuria, polydipsia, and fatigue x 3 days. Initial labs revealed severe hyperglycemia with metabolic derangement. Placed on continuous insulin infusion (0.1 U/kg/hr titrated to effect) for 18 hours until glucose <250 mg/dL and anion gap closed. IV normal saline 250 mL/hr x 24 hrs. Echocardiogram repeated given worsening dyspnea; EF unchanged from prior (38%). Diuretics held temporarily due to acute illness; restarted Day 3 at reduced dose.

Endocrinology consultation obtained Day 2. Patient counseled on insulin self-administration, carbohydrate counting, and sick-day management. Sleep medicine referral placed for OSA evaluation given apneic episodes noted on telemetry.

****Glucose monitoring during admission:****

- Fingerstick QID pre-meals and bedtime
- Random glucose Day 4: 156 mg/dL
- POC electrolytes monitored BID Days 1–2, then daily

Discharged on basal-bolus regimen with good understanding of technique. Demonstrated competency with insulin pen and glucose meter prior to discharge.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Insulin Glargine	20 units	q.d. at bedtime	Ongoing
Insulin Aspart	6–10 units	TID with meals	Ongoing
Furosemide	20 mg	q.d.	Ongoing (renal dose)
Lisinopril	10 mg	q.d.	Ongoing
Carvedilol	6.25 mg	BID	Ongoing
Atorvastatin	40 mg	q.d.	Ongoing

| Metformin | 500 mg | BID with food | Ongoing (hold if Cr >2.0) |
| Aspirin | 81 mg | q.d. | Ongoing |

****ACTIVITY & MOBILITY****

- No lifting >10 lbs
- Ambulation as tolerated; encourage 20–30 min walking daily to improve glucose control
- Resume normal ADLs gradually over 1 week

****DIET****

- American Diabetes Association (ADA) 2000 kcal/day, consistent carbohydrate diet
- Sodium restriction 2 g/day (heart failure)
- Avoid simple sugars; focus on whole grains, lean proteins, non-starchy vegetables
- Fluid restriction: 1.5 L/day